

العمر(تاريخ الميلاد) : 29-02-1964 المهنة : ربه منزل

التوقيع: (_____)

التعليق على التشخيص : -----

تتعمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

رئيس
المجلس الطبي المتخصص

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Name: حنان الحديدي عبدالحميد المنزلاوي National ID: 26402290104326	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
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المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Contrast-enhanced CT of chest, abdomen and pelvis.

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



83175026
 رقم الهوية
 1441/04
 تاريخ الميلاد
 26/03/2023
 تاريخ التسجيل



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Pat. Name: Hanan Al Hadidy Abdel Hameed Almanzalawy	Pat. ID: 10175862
Pat. Age 60 Y(s)	Pat. Sex F
Study ID: 17817	Study End Date: 04/07/2026 22:00:00
Procedure Name: PET-CT	Ref. Phy. Gistave Rosy

¹⁸F-FDG PET/CT examination

Clinical history: follow-up of a patient with history of bilateral breast neoplastic lesions (no pathology available) on palliative hormonal/targeted treatment.

Dose was injected according to body weight.

Procedure:

- Prior to administration of the radiotracer, blood glucose level was 146 mg/dl.
- CT images for attenuation and localization followed by PET images from the skull vertex to mid-thigh were obtained.
- Hepatic SUVmax reference ~ 4.6
- **Compared to the previous PET-CT study dated 07/02/2026.**

Findings:

- **Right breast:**



- **Metabolic and morphologic rather stationary course of the previously noted FDG avid multiple breast lesions**, the dominant retro-areolar lesion achieves SUVmax 3.6 compared to 3.9 and still measuring about 2.4 x 6.1 cm. Still seen merging with other axillary tail lesion (10 O'clock position) showing exceptional mild regression with SUVmax 1.9 compared to 3.4 previously.

- **Left breast:**

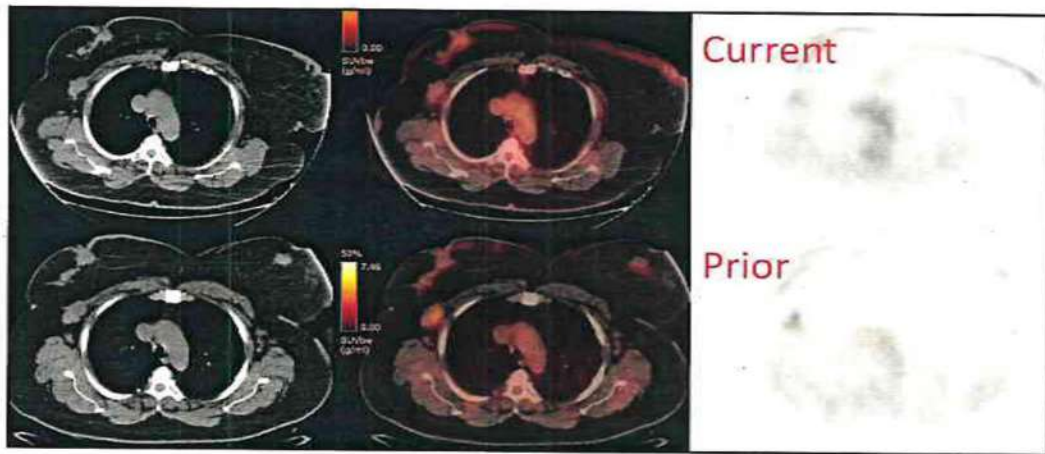
- **No significant metabolic or morphologic change** of the previously noted FDG avid left breast lesion with irregular outline and spiculated margins, currently achieving SUVmax 2.8 compared to 3.1 and still measures about 3 cm along its maximum axial dimension.

Name: حنان الحديدي عبد الحميد			
MR. : 2546			
Detailed Medical Report			
			السيد
تحتية طبية وبعد,,			
		إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
		رقم طبي:	بقسم:
ننشر بأن نوضح فيما يلي تقريراً عن حالته			
تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمتابعة وجد انه			
The patient is a 62-year-old with a history of bilateral breast cancer diagnosed in November 2023, HER2-positive (1+), initially treated with anastrozole. Disease progression was noted on a PET-CT performed on 7 February 2026, demonstrating mild progression of multiple breast lesions, involvement of axillary lymph nodes, and a right humeral lesion, prompting multidisciplinary discussion and continuation of zoledronic acid (Zometa) with anastrozole. On 25 March 2026 the patient was prescribed fulvestrant (Faslodex) and palbociclib (Ibrance) 125 mg, with the pharmacy order indicating administration on day 1 and a subsequent dose on day 15. On 8 April 2026 an electrocardiogram was ordered prior to continuation of fulvestrant, as the patient had initiated palbociclib without a baseline ECG. On 20 May 2026 physical examination revealed right-breast nipple retraction, a left-breast mass with nipple retraction, and the patient remained on fulvestrant, palbociclib 125 mg daily, and zoledronic acid. On 15 July 2026 a PET-CT performed on 4 July 2026 showed stable disease of the breast masses, right axillary lymph node, left axillary lymph node, and osseous lesions, with a slight increase in SUV of the right humeral head (6.6 vs 5.7) and the left humeral head (5.3 vs 4.5). The plan was to continue and closely follow the current regimen of fulvestrant and palbociclib 125 mg. On 12 August 2026 the patient continued zoledronic acid, fulvestrant, and palbociclib 125 mg with no significant toxicity reported. The patient has completed two cycles of fulvestrant and palbociclib to date.			
تحريرا في ,			
		التاريخ :	20-09-2026
		الوقت :	
		الطبيب المعالج :	dina makram
يعتمد ,			



• **Lymph nodes:**

- **Right axillary, mild metabolic and morphologic regression** as regard levels I and II LNs, current SUVmax 2.6 and measuring 3.4 x 2.2 cm (compared to SUVmax 3.8 and measured 3.9 x 2.4 cm at a level I LN).



- **Left axillary, rather stationary course** of the low grade active levels I and II LNs, with SUVmax 1.8 compared to 1.1 and still measures 1.2 at a left level I LN.
- Small subcentimetric **right internal mammary**, with no significant FDG uptake.
- Still noted small low grade **mediastinal** aorto-pulmonary and hilar LNs, likely non-specific.
- Still noted bilateral pelvic external iliac lymph nodes with low grade FDG uptake, likely non-specific.



AL SHROOUK SCAN

الشروق تـلاشـمة

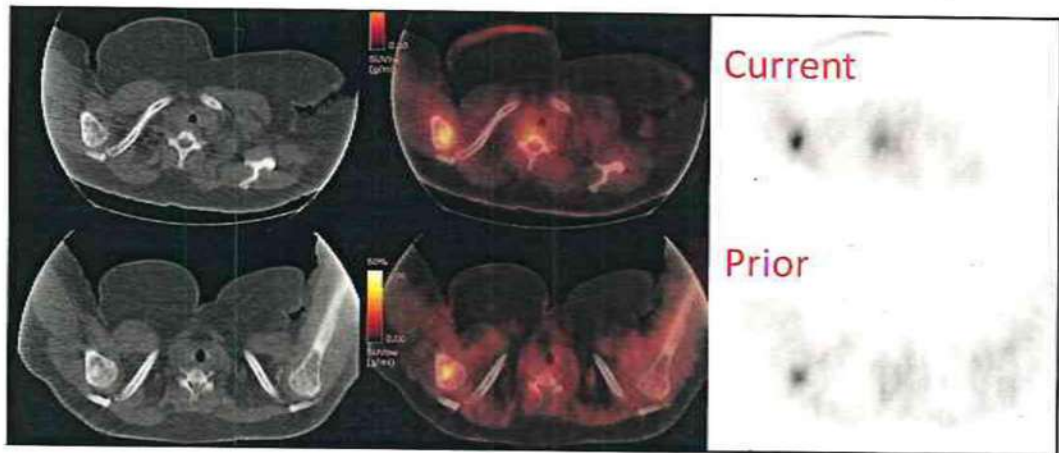
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- **Bone marrow:**

- **No significant interval metabolic changes** regarding the previously seen FDG-avid widespread osseous lesions involving the skull base, scapulae, clavicles, humeri, sternum, bilateral ribs, vertebrae, sacrum, pelvic bones, femora and tibiae.

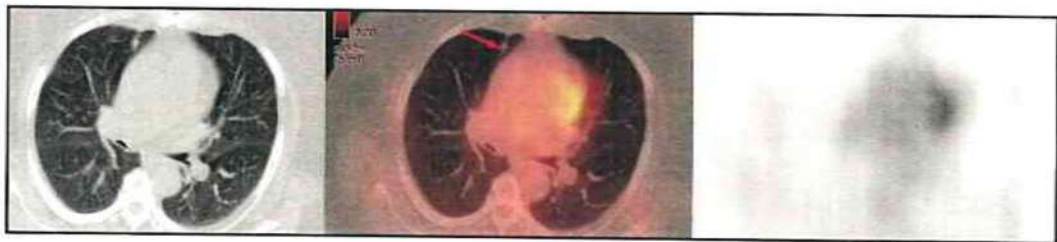
Examples include:-

- Right humeral head with SUVmax 6.6 compared to 5.4.
- CV6 with SUVmax 5.3 compared to 4.5.
- Some of the involved ribs show pathological fracture.



- **Pleuro-pulmonary:**

- **Stationary course** of the non FDG avid:-
 - Two bilateral reticulo-nodular densities reaching 12 mm at the right middle lobe, for follow-up.



The rest of the body

- **Head and Neck region:**

- **Thyroid:** still noted nodular goiter with calcification within the right lobe and patchy FDG activity still achieving SUVmax 3.6, for correlation with ultrasonography.

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الفرع الرئيسي: برج بلازا الطبي - الدور الأول - أعلى جملة ماركت - بجوار برج الأطباء - أول كوبري فيصل
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PET Scan
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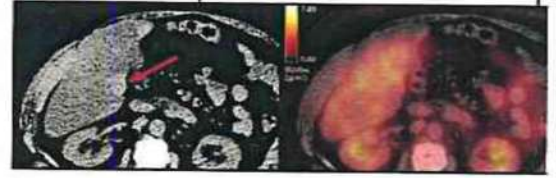
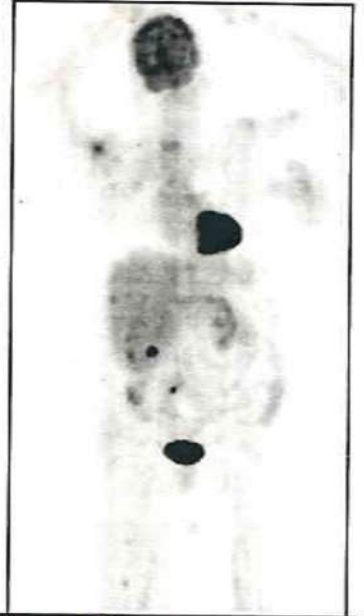
AL SHROOUK SCAN

الشروق تـلـاشـعـة



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- Normal FDG uptake within the salivary glands.
- Normal physiological uptake within the brain with no focal lesions.
- Still seen few mild active bilateral cervical lymph nodes, likely inflammatory.
- **Abdomen and pelvis:**
 - Liver: Enlarged with diffuse fatty infiltration with no definite FDG avid focal lesions. No dilated intra-hepatic biliary radicles.
 - Currently noted segment V relatively hyperdense focal area measuring 2.7 x 2 cm with isometabolic activity, likely area of fat sparing.
 - Average size spleen with no focal FDG-avid lesions.
 - Normal adrenals, pancreas, kidneys.
 - Low grade FDG uptake corresponding to the uterus.



OPINION:

***Follow-up study demonstrates overall near stationary course:

- Metabolic and morphologic rather stationary course of the both breast neoplastic lesions.
- Mild regressive course of the right axillary nodal lesions and rather stationary course of the left axillary ones.
- No significant interval metabolic changes of the widespread osseous deposits; as described.
- Stationary course of the two non metabolically active bilateral pulmonary reticulo-nodular densities, for further follow-up.

Radiologist

Dr. Dalia Ibrahim, MD

Fathey, MD

Nuclear Medicine physician

Asst. Prof. Huda

Dr. Osama Elmogy, MSc

AL SHROOUK SCAN

الشروق تـلـاشـعـة

الفرع الرئيسي: برج بلازا الطبي - الدور الأول - أعلى جملة ماركت - بجوار برج الأطباء - أول كوبري فيصل
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PET Scan
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02/21/2026

MDT FORM

Name:	Hanan * Abdelhamid *	MRN:	2546
Age:	62 Y	ID:	4470-2023
Menopausal Sta	POSTM	Phone:	01099212480
NRS SCORE:		Marital Status:	
BMI:		Family History:	
CO Morbidity:		Reason Of MDT:	

Clinical Data:

BX: IDC
ER: 8/8 PR: 8/8 HER2: +1 KI67:

R

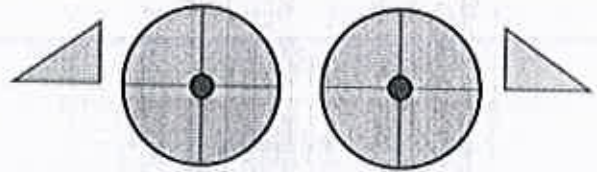
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Sonomammography:

Mets Workup:

History:

BILATERAL BREAST CANCER MET TO BONE ,LN,PLEURA,LUNG DIAGNOSED FROM 11/2023 ST FEMARA TILL NOW PET CT ON 7/2/2026 MET PROGRESSION OF MULTIPLE BREAST LESIONS PD OF AXILLARY LYMPH NODES MET PD OF RT HUMERAL LESION .FOR MDT



T N M

Recommendations:

MDT Outcome & Plan

Ibrance faslodex zometa every 3 ms

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ حنا عروت					



Patient Name	: الأستاذة / حنان الحديدي عبدالحميد المنزلاوي	Request No.	: 13003209
Patient No.	: 26402290104326	Request Date	: 14/11/2023
Path No.	: 4030/23	Verification Date	: 27/11/2023
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 59 Y	Department	: Out Patient
Referring Doc.	:	IHC No.	:
		Registered by	: marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right&left breast mass.

Nature of Specimen: Tru-cut biopsy, collected at 14/11/2023.

Technique:

Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) stained by rabbit monoclonal antibodies using Ultraview HRP DAB as chromogen and hematoxylin as counter-stain. The sections were treated against: ER clone (1E2), PR clone (SP1), Her2 clone (4B5) and Ki67 clone (30-9).

Results:

- IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 90% of tumor cells (Allred score 8/8).
- PR: Strong nuclear reaction in 90% tumor cells (Allred score 8/8).
- Her2: Faint incomplete membrane reaction in >10% tumor cells (score 1+).
- Ki67: 10% nuclear reaction of tumor cells.

Conclusion:

Bilateral breast cancer, Tru-cut biopsy:

Right breast

Invasive duct carcinoma, Grade II-III

Left breast

Invasive duct carcinoma, Grade II

PT: (on both breast cores)

• ER: Positive (score 8/8).

• PR: Positive (score 8/8).

• Her2: (score 1+).

• Ki67: Positive in 10% of tumor cells.

Verified by:

Dr. Heba Hasan

Printing Date : 27/11/2023 15:22:17

Page 1 of 2

Printed By : marwa mohamed yousse

